

Q My mother is a twin. Should I tell my doctor about that?

Yes, if your mother has a family history of nonidentical twins say so. In some cases, you may go for an ultrasound as early as five weeks to see if you are carrying more than one baby. However, about 20 to 30 percent of twin embryos seen early on become single pregnancies before the next ultrasound—known as “vanishing twin syndrome.” The causes are not known but there are no ill effects on the remaining embryo.

Q Will my symptoms be twice as bad if I'm carrying twins?

Some pregnancy symptoms may be more severe (but not twice as bad) than if you were carrying a singleton, but thankfully this is not the case for everyone. Nausea can be worse because you'll have higher hCG levels than in a single pregnancy. You may also put on more weight, and more quickly; in later pregnancy this can put added strain on your back. You might be more breathless as your babies grow

and push against your diaphragm. You are also more likely to experience swelling, heartburn, constipation, and indigestion.

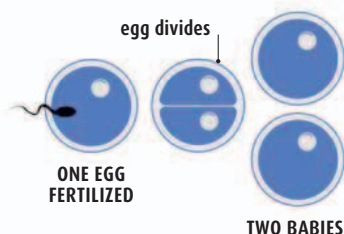
When it comes to labor and birth, having twins (or more) should not take much longer than if, under the same circumstances, you were to give birth to only one baby. Very rarely, “deferred labor” of the second baby can occur: the first baby is born, then labor stops and there is a significant gap (a day or more) before the second baby arrives. Your hospital would offer you support and careful monitoring until the second baby was born.

Q Are my twins identical or nonidentical?

You're not the only one who is curious to know the answer—your doctor will want to find out whether your twins are identical, and more importantly whether they share a placenta or even an amniotic sac.



Identity often becomes obvious as twin babies grow older.

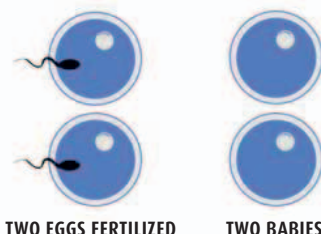


Identical twins (monozygotic) When a single egg that is fertilized by a single sperm splits in two, it forms twins that share the same DNA: identical. They are always the same sex.

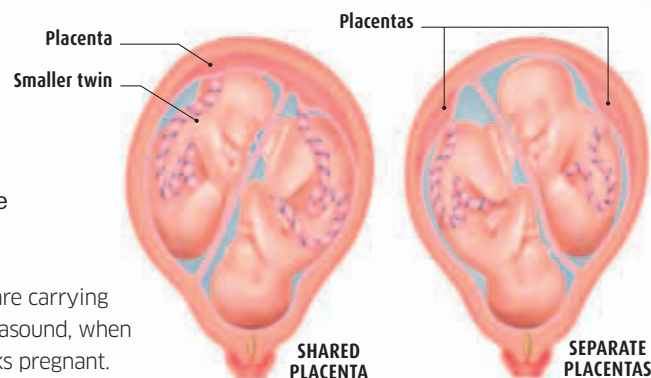
You will usually find out if you are carrying multiple babies at your first ultrasound, when you are between 8 and 14 weeks pregnant. At this ultrasound, the sonographer will assess what type of placenta the babies have (chorionicity) and what type of amniotic sac (amnionicity), since this is critical for their care.

Chorionicity and amnionicity

All nonidentical twins have separate placentas (dichorionic) and amniotic sacs because they come from two different eggs that each attach separately to the uterus (see p.33). Sometimes the two placentas can fuse, but they are still considered dichorionic. Identical twins may or may not have separate placentas and amniotic sacs, depending on how early the initial fertilized egg (zygote) divided. A split within three to four days of fertilization creates a placenta and



Nonidentical twins (dizygotic) If two eggs are fertilized by two different sperm, your twins will have different DNA: nonidentical. They may be the same sex or different sexes.



Mono- and dichorionic identical twins A shared placenta may cause unequal blood circulation, thereby restricting the growth of one twin. Babies with separate placentas, and separate blood circulation, are more likely to grow to equal size.

amniotic sac for each twin (one-third of cases); a split during days four to eight forms one shared placenta (monochorionic—the majority of identical twins) and two amniotic sacs (diamniotic); after eight days, the babies will share both a placenta and an amniotic sac (monochorionic and monoamniotic).

Twin-to-twin transfusion syndrome

A shared placenta can result in “twin-to-twin transfusion syndrome” (TTTS), when one baby “donates” blood to the other, but fails to thrive properly itself. Furthermore, the extra blood can put a strain on the receiving twin's heart. This condition occurs in about 15 percent of identical twin pregnancies. If you have monochorionic twins, you may have ultrasounds every two to three weeks from 16 weeks to keep an eye on how the babies are growing. Recent obstetric advances have improved the survival rates of twins with TTTS, with laser surgery helping to ensure a better flow of blood between the babies.